

Skin Injury - Female (Child)

Algorithm 1284 | Child (0-17 years) | Gender at birth: Female

Definition

- Qatar-localized UAT-only adult and pediatric entry pathway for an initially nonspecific skin injury, screening for major bleeding, deep or contaminated wounds, neurovascular damage, high-risk sites and safeguarding; not approved for production.
- Child patient.
- Female.

Initial assessment

1. [LOCATION] Where is the injury?
2. [OPEN_TEXT] How did it happen, and what does it look like?
3. [YES_NO] Is the bleeding under control?
4. [OPEN_TEXT] What is the exact age; is the patient pregnant or taking anticoagulants; is the injury near an eye, face, neck, chest, abdomen, genital area, joint, tendon, vessel or nerve; and are there diabetes, poor circulation, immune suppression, bite/injection, contamination, tetanus or safeguarding concerns?
5. [YES_NO] If the patient is under 18, can they speak privately now, is the accompanying caregiver considered safe, and is there an approved safe callback method if contact is lost?
6. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Burns - Thermal

Was the skin injury caused mainly by heat, flame, steam, or a hot object?

A heat-related burn belongs in the Burns - Thermal pathway.

1. Disposition level 100

If the patient is under 18, is harm occurring now, is there immediate danger from an accompanying or suspected caregiver, could the child be returned now to an unsafe person or place, or did contact end while an immediate threat remained unresolved?

Immediate danger, ongoing harm, an imminent unsafe return, or loss of contact during an unresolved immediate threat requires emergency action. Qatar 999 is the verified emergency route. Exact child-protection reporting, consent, documentation, and information-sharing requirements are local approval required.

2. Disposition level 78

If the patient is under 18 and there is no confirmed immediate danger, are they unable to speak privately, is caregiver safety uncertain, or is there another safeguarding concern requiring trained assessment?

An inability to obtain a private history, uncertainty about caregiver safety, or another safeguarding concern needs prompt trained assessment without automatically treating it as a confirmed emergency. The exact Qatar safeguarding pathway requires local approval.

3. Disposition level 100

Is bleeding uncontrolled, is there numbness or trouble moving near the wound, is the wound very large or deep, or is something stuck in it?

NHS.UK cuts and grazes guidance lists these as emergency-assessment criteria, applicable to skin injuries generally.

3. Disposition level 70

Does the wound have dirt still in it, is it swollen/red/getting more painful or leaking pus, is it larger than about 5cm, or does the caller feel generally unwell?

NHS.UK guidance lists these as reasons to seek urgent clinical review.

3. Disposition level 70

After emergency and higher-risk features are excluded, is there still a skin injury requiring wound assessment?

This broad UAT entry pathway requires in-person assessment before minor status, closure needs, tetanus status or safeguarding risk can be confirmed.

Advice

Immediate child safeguarding danger

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are local approval required. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: harm occurring now or immediate danger; imminent return to an unsafe person or place; contact lost while an immediate threat remains unresolved.

Urgent private child safeguarding assessment

- Keep questions neutral and seek a private conversation only when it is safe. Do not disclose sensitive information to a caregiver whose safety is uncertain, do not confront or alert a suspected unsafe person, and do not promise confidentiality. Arrange prompt in-person assessment through the governance-approved Qatar child safeguarding pathway and record only an approved safe callback method. If immediate danger emerges or contact is lost during an unresolved immediate threat, use the emergency branch and call Qatar 999. Exact

reporting, consent, documentation, and information-sharing rules are local approval required.

- Call Back If: private history cannot be obtained safely; caregiver safety remains uncertain; new immediate danger or loss of contact.

Emergency skin injury precautions

- Call Qatar 999 and do not drive for uncontrolled bleeding, shock or major injury. Apply firm direct pressure with a clean cloth unless an object is embedded; then press around, not on, it and do not remove it. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: bleeding will not stop; fainting, confusion, pallor or other signs of shock.

Urgent skin injury review

- Cover the wound with a clean dressing and arrange same-day in-person review through the Qatar pathway approved for UAT. Do not probe the wound, remove embedded material or use leftover antibiotics.
- Call Back If: increasing redness, pain, numbness or weakness; fever or deterioration.

In-person skin injury assessment

- Use gentle pressure for minor bleeding, cover with a clean dressing, and arrange same-day in-person assessment. Closure, tetanus and medication decisions require patient-specific clinical review.
- Call Back If: bleeding restarts or sensation/movement changes; infection or systemic illness develops.

First aid

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are local approval required. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Qatar 999 and do not drive for uncontrolled bleeding, shock or major injury. Apply firm direct pressure with a clean cloth unless an object is embedded; then press around, not on, it and do not remove it. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Cuts and grazes", <https://www.nhs.uk/conditions/cuts-and-grazes/> (page last reviewed 02 April 2026)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: 4a232b8fc7401003a8508134331752c2659b3ef3a4bf954da5fb0fdf27285c9a | Status: BLOCKED_BY_RESEARCH_GAP